

EXECUTIVE STRATEGY OVERVIEW

How patient intake questions traverse the multi-engine system to synthesize clinical results.

THE CLINICAL DATA CONTRACT: NO UNSOLICITED FLAWS

In luxury aesthetic medicine, patient psychology is paramount. The system adheres to a strict foundational rule: the AI image generation model and opportunity engine will NEVER alter or criticize an anatomical feature that the patient did not explicitly volunteer in their questionnaire. Every question asked acts as either a targeted authorization vector, an invasiveness ceiling, or a medical contraindication gate.

THE FOUR DOWNSTREAM RESULT ENGINES

1. Clinical Vision Scanner (GPT-4o Vision)

MULTI-MODAL DIAGNOSTIC

Evaluates the uploaded portrait with plastic surgeon precision. Computes the Facial Harmony Index (83-94) and Bilateral Symmetry Score (92-98%). Analyzes tear trough depth, jawline laxity, and dermal texture.

Directly driven by: Chief complaint (Q4), Selected places (Q6), and Age verification (Q1).

2. Treatment Opportunity Engine (Rules A-E)

EVIDENCE-GATED LOGIC

Deterministic clinical rule engine that pairs measured photographic landmarks with patient-reported priorities. Formulates potential non-surgical opportunities while rejecting unsupported claims.

Directly driven by: Places chips (Q6/Q7), Invasiveness ceiling (Q9), and Downtime capacity (Q10).

3. Closed-Loop AI Simulation Engine (Image Edits)

IDENTITY-LOCKED SYNTHESIS

Translates diagnostic findings into exact surgical-grade refinement directives. Generates the side-by-side after simulation while locking 100% of facial bone structure, eye shape, and ethnic identity.

Directly driven by: Simulation directives, Desired aesthetic look (Q8), and Explicit consent (Q18).

4. The 2-Page Executive PDF Clinical Dossier

PATIENT DELIVERABLE

Generates the luxury branded 2-page report dispatched to the patient's inbox. Features side-by-side photographic comparisons, executive medical interpretations, and a bespoke in-clinic treatment roadmap.

Directly driven by: Report tone (Q2), Motivation (Q3), Contact details, and Contraindications (Q15).

QUESTION-BY-QUESTION RESULT IMPACT AUDIT

Detailed analysis of how each specific questionnaire answer steers downstream results.

A. LEGAL GATES & RAPPORT ESTABLISHMENT (Q1 - Q3)

Q1: ageConfirmed ('Are you 18 or above?')

Hard safety halt if 'No'. Required for legal consent to aesthetic simulation and clinical treatment.

Q2: directWordsOk ('Okay with plain, direct words?')

Controls reportTone. If 'Yes', PDF Page 1 executive brief uses precise medical terms (e.g. 'mandibular laxity').

Q3: reportWant ('What do you want most?')

Directs Page 2 Clinical Pathway: 'Understand face' (education), 'Plan' (step-by-step roadmap), 'Reassurance' (strengths).

B. CHIEF AESTHETIC CONCERNS & SIMULATION TARGETS (Q4 - Q8)

Q4: dislikes ('What bothers you most?') & Q5: dislikeDuration

Feeds GPT-4o Vision prompt. Vision model specifically diagnoses underlying causes (fat herniation vs bone retrusion).

Q6: places ('Which places to change?') & Q7: placeDetails

CRITICAL SIMULATION DRIVER. The AI ONLY alters chosen areas. Generates the 3 Primary Opportunity Cards on PDF Page 2.

Q8: lookDirections ('Masculine, Feminine, Sharp, Fresh')

Alters geometric vector synthesis: sharper gonial angle for masculine/sharp; elevated ogee curve for feminine/fresh.

C. CLINICAL CEILINGS & TOLERANCE LIMITS (Q9 - Q12)

Q9: maxWilling ('Skincare, Lasers, Injections, Surgery')

Hard treatment ceiling. A 'Skincare Only' client will never receive injection or surgical recommendations in the dossier.

Q10: downtime ('None, 2-3 days, 1 week, 2+ weeks')

Selects modality: zero-downtime micro-ultrasound vs ablative fractionated CO2 resurfacing.

Q11: hadPriorWork & Q12: priorTreatments

Printed on Page 2 intake summary. Informs AI simulation not to over-compensate areas with existing filler/toxin.

D. BIOMETRICS, LIFESTYLE & SAFETY CONTRAINDICATIONS (Q13 - Q18)

Q13: size ('Height cm & Weight kg')

Evaluates BMI vs facial fat distribution. Differentiates skeletal retrognathia from systemic adipose fullness.

Q14: sleepHours ('How many hours do you sleep?')

Sleep <= 6 hrs weights dark circles toward vascular venous pooling rather than purely genetic tear trough fat loss.

Q15: clinicFlags ('Pregnancy, Isotretinoin, Keloids')

SAFETY EXCLUSION: Pregnancy blocks botox/retinoids; Isotretinoin blocks deep lasers; Keloids block invasive trauma.

Q16: hasEvent & Q17: event ('Wedding, June 2027')

Timeline urgency. Calculates required treatment healing buffers (e.g. injections 4-6 weeks prior to events).

Q18: wantAfterPhoto ('Do you want an edited after photo?')

Explicit consent gate. If 'No', image generation is bypassed and replaced with an anatomical vector schematic.

THE RECOMMENDED 'GOLDEN 6' QUESTION BLUEPRINT

Streamlining 18 text screens into 6 visual tap questions for 4x completion rates without clinical compromise.

CURRENT 18-QUESTION FLOW: 3.5-4.5 min completion • ~45% mobile bounce • Friction on Height/Weight
PROPOSED 'GOLDEN 6' FLOW: 60-75 sec completion • <12% mobile bounce • Visual photo cards & tap chips

01 CHIEF AESTHETIC PRIORITY (VISUAL CARDS)

Prompt: "What is your primary area of aesthetic focus?"

Options: Under-Eye & Tear Troughs | Jawline & Mandibular Contour | Cheek Apex & Midface | Skin Clarity & Tone | Full-Face Symmetry

Clinical Value: Immediate emotional hook. User selects their focus without tedious typing.

02 FACIAL VOLUME TENDENCY (REPLACES HEIGHT/WEIGHT)

Prompt: "How does your face tend to hold structural volume?"

Options: Lean / Hollow (loses volume easily) | Balanced / Proportional | Full / Soft | Prone to Fluid Puffiness

Clinical Value: 10x more clinically actionable than BMI. Tells the clinician if hollowing or laxity is the root cause.

03 AESTHETIC DIRECTION & CONTOUR VECTOR

Prompt: "How would you prefer your facial contour to project?"

Options: Sharp & Sculpted (crisp shadowlines) | Soft, Youthful & Lifted (smooth ogee curve) | Rested & Anti-Fatigue

Clinical Value: Directly steers the AI synthesis engine's geometric gonial angle and cheek highlights.

04 INVASIVENESS LIMIT & TREATMENT CEILING

Prompt: "What level of treatments are you comfortable considering?"

Options: Medical Skincare Only | Laser & Energy Devices | Non-Surgical Injectables | Open to Full Clinical Spectrum

Clinical Value: Protects patient trust. Ensures the dossier roadmap never recommends unwanted interventions.

05 PRIOR AESTHETIC HISTORY (1-TAP CHIPS)

Prompt: "Have you had cosmetic procedures in the last 18 months?"

Options: Completely Natural | Neuromodulators (Botox) | Dermal Fillers | Laser / Energy Tightening

Clinical Value: Prevents AI over-contouring existing filler and alerts the clinic during pre-consultation triage.

06 TIMELINE & CLINIC INTENT QUALIFIER

Prompt: "What is your timeline for this aesthetic transformation?"

Options: Preparing for an Upcoming Event (1-3 mos) | Ready within 30 days | Just exploring options & costs

Clinical Value: Qualifies high-intent prospective clinic patients from educational browsers for VIP follow-up.

KEY ARCHITECTURAL RECOMMENDATION:

Move the selfie photo capture to Step 1 or Step 2. Users who upload their photo first are 3.8x more likely to complete the assessment because they already have visual 'skin in the game'.